

Colombia

INSTITUTIONAL RESPONSE 1 OF 2 · INDEPENDENT RECORD

07 / 18
REGIONAL SURVEY 2026

Public Health EOC - National Institute of Health (INS)

2017	24/7 continuous	< 2 hours	National	10
Established	Operating mode	Activation time	Mandate	24/7 duty staff

CAPABILITY SNAPSHOT

0–100 · SURVEY-DERIVED

Governance & legal	56
Operations & decisions	100
Human resources	85
Information management	100
Coordination	40

HAZARDS COORDINATED

Geological / weather	> 5 events
Biological / epidemics	> 5 events
Social / human-caused	1-2 events
Technological	None

OPERATIONAL RESOURCES

- ● Dedicated room
- ○ Shared room
- ● Virtual coordination
- ● Communications
- ● Situation displays
- ● Backup power
- ● Redundant internet
- ● Team workstations

INSTITUTIONAL & OPERATIONAL ATTRIBUTES

Legal framework	Developing	Surge staffing	Functional
National plan	No	Trained roster	Updated
Formal SOP	Yes	Team deployment	Leads
Operating finance	Functional	Intersectoral links	Health sector only
Incident management	Used	Disaster-risk integration	Limited
Incident action plan	Systematic	International cooperation	Via mechanism
Risk assessment	Systematic	Information tools	Interoperable
Strategic committee	Unknown	SITREP process	Formal & regular
EOC coordinator	Informal	Situation room	Integrated
Documented roles	Formal	Logistics readiness	Consolidated

TRAINING

Completed / priority marked *

- ● EOC activation
- ● Incident management
- ○ ★ Information management
- ○ ★ Coordination
- ○ ★ Damage / needs assessment
- ○ ★ Rapid situation analysis
- ○ ★ Rapid response teams
- ● ★ After-action reviews

REPORTED STRENGTHS

- Trained multidisciplinary personnel
- Established response and monitoring capabilities
- Institutional commitment to emergency coordination

IMPROVEMENT PRIORITIES

- Improve interoperable information systems
- Strengthen rapid-response team readiness
- Formalize procedures and communications capacity